



February 20, 2026

Thomas Keane, MD, MBA
Department of Health and Human Services (HHS)
Assistant Secretary for Technology Policy (ASTP)
Office of the National Coordinator for Health Information Technology (ONC)
Attention: Request for Information: HHS Health Sector Artificial Intelligence (AI) RFI
Mary E. Switzer Building, Mail Stop: 7033A
330 C Street SW
Washington, DC 20201

Re: HHS Health Sector AI RFI

Submitted electronically to: <http://www.regulations.gov>

Dear Assistant Secretary Keane:

Health Level Seven (HL7) International welcomes the opportunity to provide feedback on the HHS Health Sector AI RFI and what HHS can do to accelerate the adoption and use of AI as part of clinical care. This letter includes perspectives from our leadership and Policy Advisory Committee.

Overall, HL7 encourages ASTP/ONC to actively engage our organization as its plan to accelerate the adoption and use of AI as part of clinical care is constructed and deployed. HL7 is a rich resource with:

- (1) Leadership steeped in AI, including our CEO Rachel Dunscombe and dedicated Chief AI Officer Dr. Daniel Vreeman, DPT;
- (2) Recent and relevant legacy HL7 AI Taskforce and Initiative work;

- (3) AI public-private sector policy convenings, such as HL7 policy conference on healthcare interoperability, genomics and AI. HL7 also has critical Work Groups and Accelerators focused on clinical care issues; and
- (4) World-leading and globally relevant expertise regarding standards that incorporate appropriate rules and usage of AI.

HL7 is the global authority on healthcare interoperability and a critical leader developing open standards that drive digital health innovation. As such, we stand ready to support ASTP/ONC in better understanding the promises, complexities and challenges at the crossroads of standards, interoperability, clinical care, policy and AI.

As a Standards Development Organization (SDO) with broad-ranging public-private sector membership, HL7 has a front row seat to many complexities and more serious issues emerging with AI use in healthcare and clinical care that we wish to highlight for HHS in response to this RFI such as:

- Currently, both health information technology systems and users do not always know what data was generated or modified by an AI tool or what tool was used-- therefore the data can appear falsely, as if it was generated only by a clinical user.
- AI models can generate actions around care delivery that are incorrect (like inaccurate categorization of cardiac or cancer risk). An optimal way to identify those mistakes within medical records and correct them must be identified. This kind of metadata is also key for safety.
- There are not requirements or transparency around the monitoring of known AI issues like model degeneration and hallucination (confident but false, fabricated or nonsensical information generated by AI models).

Specific to the effort to further understand what HHS can do to accelerate the adoption and use of AI as part of clinical care; we highlight the following expert HL7 resources:

HL7 Artificial Intelligence (AI) Office

HL7 has an AI Office focused on establishing foundational standards for safe, trustworthy and

scalable AI in healthcare and convening the global community driving this transformation. The new office keeps HL7 at the forefront of healthcare's AI revolution, creating frameworks that ensure emerging technologies are trusted, explainable, interoperable and scalable across clinical, operational and research settings worldwide. It also ensures that healthcare interoperability and AI have a productive and symbiotic relationship, maximizing the promise of interoperability for better care, while continuing and expanding healthcare efficiencies. These are significant. The Organization for Economic Co-operation and Development estimates that the value of enabling interoperable sharing of health data is between 2.70% to 6.64% of health system expenditure and may be as high as 17%.

HL7 serves as the trusted global convener for responsible, standards-driven AI innovation—ensuring these transformative technologies deliver on their promise to improve health for all. Leading this initiative, HL7 has appointed Daniel Vreeman, DPT, as its first Chief AI Officer (CAIO). A globally recognized biomedical informatics expert, Dr. Vreeman expanded his role as Chief Standards Development Officer to drive HL7's comprehensive AI strategy, including the HL7 AI Challenge, anti-fraud initiatives, and collaborations with regulators and industry partners globally. In our newest AI partnership, HL7 has become a founding member of the Vigilant AI Monitoring and Operations System (VAMOS) Collaborative --a consortium of leading academic medical centers and industry partners focused on AI governance and algorithmovigilance --through open standards and platforms.

In January 2026, HL7 conducted the first ballot of a new specification: the [AI Transparency on FHIR Implementation Guide](#). This specification will provide crucial implementation guidance for the key aspects of documenting when FHIR resources have been created, updated, or influenced by AI. This type of data-level transparency-- the practice of openly disclosing how AI systems work, including their data sources, logic and decision-making processes--is essential for establishing trustworthy AI systems.

HL7 AI Taskforce

HL7 had previously convened a dedicated AI Initiative and Taskforce. We worked in collaboration with multiple partners -- in both the public and private sectors -- to develop white papers and guidance to ensure the reliability, safety and security of AI. HL7 is happy to share important lessons learned in these efforts. A May 2025 report specific to AI, healthcare, payment integrity and fraud can be found at:

<https://info.hl7.org/ai-report>

HL7 Work Groups, Accelerators and Other Efforts

HL7 Work Groups, FHIR Accelerators and special initiatives are conducting cutting-edge healthcare, interoperability and standards work very relevant to the HHS Health Sector AI RFI and accelerating the adoption and use of AI as part of clinical care. A few cross-cutting topics these groups address with direct relevance for AI use cases include: data quality, provenance, consent, metadata labels and integrating external decision support applications into clinical workflows.

HL7 executive and policy leaders can facilitate structured, focused dialogue with HHS and these HL7 groups that have practical and field-leading expertise. Full listings for our HL7 Work Groups, Accelerators and information on our Policy Advisory Committee can be accessed at:

<http://www.hl7.org/Special/committees/index.cfm?ref=nav>

<https://www.hl7.org/about/fhir-accelerator/>

<https://confluence.hl7.org/spaces/PAC/pages/76159333/Policy+Advisory+Committee+Home>

Specific Comments

HL7's open, consensus-based standards serve as the essential digital foundation that enables innovative companies and startups to securely access, analyze and derive insights from healthcare data, accelerating the development of AI-powered solutions that can dramatically improve patient outcomes while reducing costs across the U.S. healthcare system. By supporting HL7 through the HHS AI and related initiatives, the federal government can unlock unprecedented market-driven innovation through ensuring standardized, interoperable health data exchange that allows new entrants and established players to compete based on the value they deliver, rather than being blocked by proprietary data silos. A strategic investment in HL7 standards would catalyze a new wave of American healthcare entrepreneurship, enabling data-driven and AI-backed solutions to flourish in a competitive marketplace that rewards improved health outcomes, ultimately making healthcare more accessible, efficient and effective for all Americans. These comments relate to the general RFI request for information and RFI question 8, *Where would enhanced interoperability widen market opportunities, fuel research, and accelerate the development of AI for clinical care?*

Regarding insight on high priority policy items as articulated in RFI question 2, HL7 recommends that the federal government should adopt voluntary consensus healthcare standards broadly in regulatory and sub-regulatory policies that govern the use of AI. This includes standards for creating, deploying, operating, measuring, reporting and ensuring the needed transparency, safety and privacy in use of AI. Adopting voluntary consensus standards that comply with the World Trade Organization's Agreement on Technical Barriers to Trade, such as American National Standards Institute (ANSI)-accredited American National Standards, is the optimal way for regulation to support the competitiveness of United States innovation in AI. HL7 emphasizes that the best organization to take on an AI standards effort should be a broad based and ANSI-accredited one. Our organization is immediately ready and willing to provide the necessary and critical AI leadership HHS needs at this time. HL7 has flagship AI initiatives underway and is leading dialogue and progress on the four key objectives in ANSI's *The United States Standards Strategy 2025* as they relate to AI which are to:

1. Strengthen U.S. commitment to international standards by leading strategic collaboration and defending against geopolitical manipulation;
2. Demonstrate standards' broad value by expanding education about how standards drive innovation, safety and market access;
3. Make standards development more efficient by embracing process innovation and training the next generation of standards leaders; and
4. Address opportunities and challenges head-on through active participation in developing emerging technology standards and sustained investment in the standards ecosystem.

HL7 also notes that the need for AI transparency is paramount because clinical care operates in a high-stakes, regulated, trust-dependent environment where opaque decision-making can directly affect patient safety, equity and legal accountability.

Lastly, HL7 supports federal preemption of conflicting state laws to prevent undue burden and ensure consistent application and regulation of AI. When states impose conflicting standards, measures and other requirements, then significant burdens can prevent achieving the potential benefits of AI. Such pre-emption is highlighted in documents such as the December 2025 Executive Order *Ensuring a National Policy Framework for Artificial Intelligence*.

Should you have any questions about our attached comments, please contact Rachel Dunscombe, Chief Executive Officer of Health Level Seven International at ceo@HL7.org. We look forward to continuing this discussion and offer our assistance to you on this topic that is critical to the future of our nation's healthcare.

Sincerely,

Rachel Dunscombe
Chief Executive Officer
HL7 International

Caroline Macumber
Board of Directors, Chair
HL7 International